



Bill Number: S.B. 1628

Angius Floor Amendment

Reference to: printed bill

Amendment drafted by: Leg. Council

FLOOR AMENDMENT EXPLANATION

1. Applies reporting requirements relating to claims denial practices to health care services plans, rather than health care insurers.
2. Requires claims denial and prior authorization reports to specify the total number of adverse determinations that were partially reversed on appeal and the total number of adverse determinations that were completely reversed on appeal.
3. Requires the outlined reports to specify the number of appeals received for specified levels of review and the total number of claims that were downcoded.
4. Specifies that the reporting requirement for the top 10 denied services applies to both inpatient and outpatient services claims and prior authorization requests that were denied.
5. Allows the Director of the Department of Insurance and Financial Institutions to adopt rules to implement the reporting requirements.
6. Makes technical and conforming changes.

ANGIUS FLOOR AMENDMENT
SENATE AMENDMENTS TO S.B. 1628
(Reference to printed bill)

Amendment instruction key:

[GREEN UNDERLINING IN BRACKETS] indicates text added to statute or previously enacted session law.

[Green underlining in brackets] indicates text added to new session law or text restoring existing law.

~~[GREEN STRIKEOUT IN BRACKETS]~~ indicates new text removed from statute or previously enacted session law.

~~[Green strikeout in brackets]~~ indicates text removed from existing statute, previously enacted session law or new session law.

<<Green carets>> indicate a section added to the bill.

~~<<Green strikeout in carets>>~~ indicates a section removed from the bill.

1 The bill as proposed to be amended is reprinted as follows:

2 Section 1. Title 20, chapter 15, article 1, Arizona Revised
3 Statutes, is amended by adding section 20-2512, to read:

4 20-2512. Health care services plans; claims denial practices;
5 reporting requirements; definitions

6 A. ON OR BEFORE JULY 1, 2027 AND EACH JULY 1 THEREAFTER, A HEALTH
7 CARE ~~[INSURER]~~ [SERVICES PLAN] SHALL REPORT TO THE DEPARTMENT ON A FORM
8 PRESCRIBED BY THE DEPARTMENT THE FOLLOWING AGGREGATED DATA THAT RELATES TO
9 THE HEALTH CARE ~~[INSURER'S]~~ [SERVICES PLAN'S] CLAIMS DENIAL PRACTICES FOR
10 THE PRIOR ~~[PLAN]~~ YEAR:

11 1. THE TOTAL NUMBER OF CLAIMS REQUESTS, INCLUDING THE TOTAL NUMBER
12 OF CLAIMS REQUESTS THAT WERE NOT SUBMITTED ELECTRONICALLY.

13 2. THE TOTAL NUMBER OF CLAIMS REQUESTS THAT WERE [PARTIALLY] DENIED
14 [AND THE TOTAL NUMBER OF CLAIMS REQUESTS THAT WERE COMPLETELY DENIED].

15 3. THE TOTAL NUMBER OF APPEALS THAT WERE RECEIVED [FOR ALL OF THE
16 FOLLOWING LEVELS OF REVIEW:

17 (a) INITIAL APPEAL.

18 (b) VOLUNTARY INTERNAL APPEAL.

19 (c) EXTERNAL INDEPENDENT REVIEW.

20 (d) EXPEDITED MEDICAL REVIEW.

21 (e) EXPEDITED APPEAL.

22 (f) EXPEDITED EXTERNAL INDEPENDENT REVIEW].

23 4. THE TOTAL NUMBER OF ADVERSE DETERMINATIONS [THAT WERE PARTIALLY
24 REVERSED ON APPEAL AND THE TOTAL NUMBER OF ADVERSE DETERMINATIONS] THAT
25 WERE REVERSED ON APPEAL.

26 [5. THE TOTAL NUMBER OF CLAIMS THAT WERE COMPLETELY DOWNCODED.]

27 ~~[5.]~~ [6.] THE TOP TEN [INPATIENT AND TOP TEN OUTPATIENT] SERVICES
28 [CLAIMS] THAT WERE DENIED IN EACH OF THE FOLLOWING CATEGORIES:

29 (a) MEDICAL AND SURGICAL PROCEDURES.

1 (b) DIAGNOSTIC TESTS AND DIAGNOSTIC IMAGES.

2 (c) BEHAVIORAL HEALTH SERVICES.

3 (d) ORTHOPEDIC SERVICES.

4 ~~[(e) OUTPATIENT SERVICES.]~~

5 ~~6.]~~ [7.] THE TOP FIVE REASONS WHY CLAIMS REQUESTS WERE DENIED.

6 B. ON OR BEFORE OCTOBER 31, 2027 AND EACH OCTOBER 1 THEREAFTER, THE
7 DEPARTMENT SHALL:

8 1. AGGREGATE THE DATA THAT IS COLLECTED UNDER SUBSECTION A OF THIS
9 SECTION INTO A STANDARD REPORT. THE REPORT MUST SEPARATE EACH HEALTH CARE
10 ~~[INSURER]~~ [SERVICES PLAN] THAT SUBMITTED DATA BY NAME AND MUST BE WRITTEN
11 IN EASILY UNDERSTANDABLE LANGUAGE.

12 2. POST THE REPORT ON THE DEPARTMENTS' PUBLICLY ACCESSIBLE WEBSITE.

13 3. MAINTAIN AT LEAST THREE YEARS OF REPORTS ON THE DEPARTMENT'S
14 PUBLICLY ACCESSIBLE WEBSITE.

15 4. SEND A COPY OF THE REPORT TO THE SPEAKER OF THE HOUSE OF
16 REPRESENTATIVES AND THE PRESIDENT OF THE SENATE.

17 [C. THE DIRECTOR MAY ADOPT RULES TO IMPLEMENT THIS SECTION.]

18 D. FOR THE PURPOSES OF THIS SECTION:

19 1. "DOWNCODE" MEANS THE UNILATERAL ALTERATION BY A HEALTH CARE
20 INSURER OF THE LEVEL OF EVALUATION AND MANAGEMENT SERVICE CODE OR OTHER
21 SERVICE CODE THAT WAS SUBMITTED ON A CLAIM AND THAT RESULTED IN A LOWER
22 PAYMENT.

23 2. "HEALTH CARE SERVICES PLAN" HAS THE SAME MEANING PRESCRIBED IN
24 SECTION 20-3401.]

25 Sec. 2. Section 20-3102, Arizona Revised Statutes, is amended to
26 read:

27 20-3102. Timely payment of health care providers' claims;
28 grievances

29 A. A health care insurer shall adjudicate any clean claim from a
30 contracted or noncontracted health care provider relating to health care
31 insurance coverage within thirty days after the health care insurer
32 receives the clean claim or within the time period specified by contract.
33 Unless there is an express written contract between the health care
34 insurer and the health care provider that specifies the period in which
35 approved claims shall be paid, the health care insurer shall pay the
36 approved portion of any clean claim within thirty days after the claim is
37 adjudicated. If the claim is not paid within the thirty-day period or
38 within the time period specified in the contract, the health care insurer
39 shall pay interest on the claim at a rate that is equal to the legal rate.
40 Interest shall be calculated beginning on the date that the payment to the
41 health care provider is due.

42 B. If the claim is not a clean claim and the health care insurer
43 requires additional information to adjudicate the claim, the health care
44 insurer shall send a written request for additional information to the
45 contracted or noncontracted health care provider, enrollee or third party
46 within thirty days after the health care insurer receives the claim. The
47 health care insurer shall notify the contracted or noncontracted health

1 care provider of all of the specific reasons for the delay in adjudicating
2 the claim. The health care insurer shall record the date it receives the
3 additional information and shall adjudicate the claim within thirty days
4 after receiving all the additional information. The health care insurer
5 shall also pay the approved portion of the adjudicated claim within the
6 same thirty-day period allowed for adjudication or within the time period
7 specified in the provider's contract. If the health care insurer fails to
8 pay the claim as prescribed in this subsection, the health care insurer
9 shall pay interest on the claim in the manner prescribed in subsection A
10 of this section.

11 C. A health care insurer shall not delay the payment of clean
12 claims to a contracted or noncontracted provider or pay less than the
13 amount agreed to by contract to a contracted health care provider without
14 reasonable justification.

15 D. A health care insurer shall not request information from a
16 contracted or noncontracted health care provider that does not apply to
17 the medical condition at issue for the purposes of adjudicating a clean
18 claim.

19 E. A health care insurer shall not request a contracted or
20 noncontracted health care provider to resubmit claim information that the
21 contracted or noncontracted health care provider can document it has
22 already provided to the health care insurer unless the health care insurer
23 provides a reasonable justification for the request and the purpose of the
24 request is not to delay the payment of the claim.

25 F. A health care insurer shall establish an internal system for
26 resolving payment disputes and other contractual grievances with health
27 care providers. The director may review the health care insurer's
28 internal system for resolving payment disputes and other contractual
29 grievances with health care providers. Each health care insurer shall
30 maintain records of health care provider grievances. Semiannually each
31 health care insurer shall provide the director with a summary of all
32 records of health care provider grievances received during the prior six
33 months. The records shall include at least the following information:

34 1. The name and any identification number of the health care
35 provider who filed a grievance.

36 2. The type of grievance.

37 3. The date the insurer received the grievance.

38 4. The date the grievance was resolved.

39 G. On review of the records, if the director finds a significant
40 number of grievances that have not been resolved, the director may examine
41 the health care insurer.

42 H. This section does not require or authorize the director to
43 adjudicate the individual contracts or claims between health care insurers
44 and health care providers.

1 I. On or before ~~August~~ OCTOBER 1 of each year, the director shall
2 post a report on the department's publicly accessible website that
3 includes the information prescribed in subsection F of this section for
4 the prior fiscal year and that includes:

- 5 1. The total number of grievances received.
- 6 2. The average time to resolve a grievance.
- 7 3. The percentage of grievances where a health care insurer's
8 decision was overturned.

9 J. Except in cases of fraud, a health care insurer or contracted or
10 noncontracted health care provider shall not adjust or request adjustment
11 of the payment or denial of a claim more than one year after the health
12 care insurer has paid or denied that claim. If the health care insurer
13 and health care provider agree through contract on a length of time to
14 adjust or request adjustment of the payment of a claim, the health care
15 insurer and health care provider must have the same length of time to
16 adjust or request adjustment of the payment of the claim. If a claim is
17 adjusted, neither the health care insurer nor the health care provider
18 shall owe interest on the overpayment or underpayment resulting from the
19 adjustment, as long as the adjusted payment is made or recoupment taken
20 within thirty days of the date of the claim adjustment.

21 K. This article does not apply to licensed health care providers
22 who are salaried employees of a health care insurer.

23 L. If a contracted or noncontracted health care provider files a
24 claim or grievance with a health care insurer that has changed the
25 location where providers were instructed to file claims or grievances, the
26 health care insurer shall, for ninety days following the change:

27 1. Consider a claim or grievance delivered to the original location
28 properly received.

29 2. Following receipt of a claim or grievance at the original
30 location, promptly notify the health care provider of the change of
31 address through mailed written notice or some other written communication.

32 M. This section does not preclude a health care provider, with
33 written informed consent of the patient, from collecting monies for a
34 medical service that is either:

35 1. Not covered under the insurance policy.

36 2. Medically necessary and a payment on the claim was not made due
37 to a denial on the basis of frequency or a disallowance on the basis of
38 frequency. For the purposes of this paragraph, a provider is limited to
39 the rates prescribed by that provider's fee schedule.

40 N. Any claim that is subject to article 2 of this chapter is not
41 subject to this article.

1 Sec. 3. Title 20, chapter 26, article 1, Arizona Revised Statutes,
2 is amended by adding section 20-3408, to read:

3 20-3408. Health care services plans: prior authorization
4 practices; reporting requirements; rules

5 A. ON OR BEFORE JULY 1, 2027 AND EACH JULY 1 THEREAFTER, A HEALTH
6 CARE SERVICES PLAN SHALL REPORT TO THE DEPARTMENT ON A FORM PRESCRIBED BY
7 THE DEPARTMENT THE FOLLOWING AGGREGATED DATA THAT RELATES TO THE HEALTH
8 CARE SERVICES PLAN'S PRIOR AUTHORIZATION PRACTICES FOR THE PRIOR ~~[PLAN]~~
9 YEAR:

10 1. THE TOTAL NUMBER OF PRIOR AUTHORIZATION REQUESTS, INCLUDING THE
11 TOTAL NUMBER OF PRIOR AUTHORIZATION REQUESTS THAT WERE NOT SUBMITTED
12 ELECTRONICALLY.

13 2. THE TOTAL NUMBER OF PRIOR AUTHORIZATION REQUESTS THAT WERE
14 ~~[PARTIALLY] DENIED [AND THE TOTAL NUMBER OF PRIOR AUTHORIZATION REQUESTS~~
15 ~~THAT WERE COMPLETELY DENIED]~~.

16 3. THE TOTAL NUMBER OF APPEALS THAT WERE RECEIVED ~~[FOR ALL OF THE~~
17 ~~FOLLOWING LEVELS OF REVIEW:~~

18 (a) ~~INITIAL APPEAL.~~

19 (b) ~~VOLUNTARY INTERNAL APPEAL.~~

20 (c) ~~EXTERNAL INDEPENDENT REVIEW.~~

21 (d) ~~EXPEDITED MEDICAL REVIEW.~~

22 (e) ~~EXPEDITED APPEAL.~~

23 (f) ~~EXPEDITED EXTERNAL INDEPENDENT REVIEW]~~.

24 4. THE TOTAL NUMBER OF ADVERSE DETERMINATIONS ~~[THAT WERE PARTIALLY~~
25 ~~REVERSED ON APPEAL AND THE TOTAL NUMBER OF ADVERSE DETERMINATIONS]~~ THAT
26 WERE ~~[COMPLETELY]~~ REVERSED ON APPEAL.

27 5. THE TOP TEN ~~[INPATIENT AND TOP TEN OUTPATIENT]~~ SERVICES ~~[PRIOR~~
28 ~~AUTHORIZATION REQUESTS]~~ THAT WERE DENIED IN EACH OF THE FOLLOWING
29 CATEGORIES:

30 (a) MEDICAL AND SURGICAL PROCEDURES.

31 (b) DIAGNOSTIC TESTS AND DIAGNOSTIC IMAGES.

32 (c) BEHAVIORAL HEALTH.

33 (d) ORTHOPEDIC SERVICES.

34 (e) OUTPATIENT SERVICES.

35 6. THE TOP FIVE REASONS WHY PRIOR AUTHORIZATION REQUESTS WERE
36 DENIED.

37 7. THE AVERAGE AND MEDIAN TIME THAT ELAPSED BETWEEN THE SUBMISSION
38 OF A REQUEST AND A DETERMINATION BY THE ISSUER FOR STANDARD PRIOR
39 AUTHORIZATIONS.

40 8. THE AVERAGE AND MEDIAN TIME THAT ELAPSED BETWEEN THE SUBMISSION
41 OF A REQUEST AND A DETERMINATION BY THE ISSUER FOR EXPEDITED PRIOR
42 AUTHORIZATIONS.

43 B. ON OR BEFORE OCTOBER 31, 2027 AND EACH OCTOBER 1 THEREAFTER, THE
44 DEPARTMENT SHALL:

45 1. AGGREGATE THE DATA THAT IS COLLECTED UNDER SUBSECTION A OF THIS
46 SECTION INTO A STANDARD REPORT. THE REPORT MUST SEPARATE EACH HEALTH CARE

1 SERVICES PLAN THAT SUBMITTED DATA BY NAME AND MUST BE WRITTEN IN EASILY
2 UNDERSTANDABLE LANGUAGE.

3 2. POST THE REPORT ON THE DEPARTMENTS' PUBLICLY ACCESSIBLE WEBSITE.

4 3. MAINTAIN AT LEAST THREE YEARS OF REPORTS ON THE DEPARTMENT'S
5 PUBLICLY ACCESSIBLE WEBSITE.

6 4. SEND A COPY OF THE REPORT TO THE SPEAKER OF THE HOUSE OF
7 REPRESENTATIVES AND THE PRESIDENT OF THE SENATE.

8 [C. THE DIRECTOR MAY ADOPT RULES TO IMPLEMENT THIS SECTION.]

9 Sec. 4. Stakeholders meeting; report

10 A. On or before July 1, 2032, the department of insurance and
11 financial institutions shall convene a stakeholders meeting that includes
12 health care insurers, health care services plans, health care institutions
13 that are regulated by the department of health services, health care
14 providers who are licensed under title 32, Arizona Revised Statutes,
15 businesses and consumers to determine the quality, relevance and
16 usefulness of the data that was reported pursuant to sections 20-2512 and
17 20-3408, Arizona Revised Statutes, as added by this act.

18 B. On or before October 31, 2032, the department of insurance and
19 financial institutions shall submit a report to the governor, the
20 president of the senate and the speaker of the house of representatives
21 with recommendations to amend or repeal or to make no changes to, sections
22 20-2512 and 20-3408, Arizona Revised Statutes, as added by this act.

23 Sec. 5. Retroactivity

24 Section 20-3102, Arizona Revised Statutes, as amended by this act,
25 applies retroactively to from and after June 30, 2026.

26 Enroll and engross to conform

27 Amend title to conform

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